

BTK: 3 molecules stopped early, all small molecules

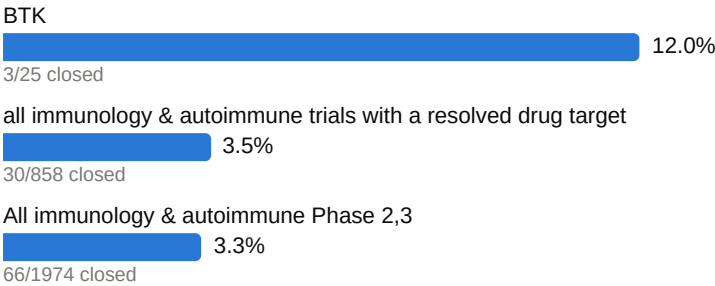
3 stopped trials, but 3 molecules — all small molecules: Elsubrutinib, Fenebrutinib and Poseltinib. 1 of the stops were the trial's own verdict and 2 followed a decision taken elsewhere. Against 3.5% for all immunology & autoimmune trials with a resolved drug target and 3.3% across all immunology & autoimmune Phase 2,3 trials in the same window. The rate counts trials that stopped early for efficacy, safety or benefit–risk reasons. Trials that ran to completion and missed their primary endpoint are counted separately below, never in the rate.

12.0% 3 of 25 closed trials (95% CI 4.2%–30.0%)	2 / 0 / 0 efficacy / safety / both (adds to 3)	62.5% of 40 trials have closed · 15 still open or unresolved	10.1% stopped for a biological reason within 3 years of starting (95% CI 3.4%–28.0%; 10 still at risk; 1.9% area-wide)
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Completed and missed the primary endpoint. Of 21 completed trials in this segment, 9 posted a primary result on ClinicalTrials.gov that can be read — the sponsor's own comparison held to the sponsor's own threshold, or its statement that the endpoint was missed: 4 missed and 3 met, 2 split across co-primary endpoints. Counted separately and never in the rate above; a floor, not a rate. 4 of the 21 completed trials that finished more than a year ago have posted no results at all — a gap in what can be known, not a sign of failure.

How this compares



What is in this class: 18 distinct experimental drugs across 40 trials (small molecule 40, unknown 1). Most tested: ZANUBRUTINIB (8), ORELABRUTINIB (6), FENEBRUTINIB (4), RITLECITINIB (4), RILZABRUTINIB (4). A class groups drugs by what they act on; check that the grouping is one you would make before reading the rate as a property of the mechanism.

How much does this rest on one decision? The 3 stops came from 3 sponsor-asset programmes across 3 sponsors; the largest (Roche) contributed 1; removing that programme's trials from both sides leaves 9.5%. Screened alongside 28 other segments in this area, it does not survive a 10% false-discovery correction (q=1, Benjamini-Yekutieli). Read it as a screen worth checking against the underlying trials.

Bars show the share of closed trials stopped for biological reasons. Confidence intervals overlap where sample sizes are small — read the counts, not just the bars.

By start cohort

Trials started	Stops	Closed	Rate	95% CI
2015–2017	1	11	9.1%	1.6%–37.7%
2018–2020	2	12	16.7%	4.7%–44.8%

Where the stops sit

Efficacy / futility: 2, Benefit–risk: 1.
Sponsors with most stops: AbbVie (1), Roche (1), Eli Lilly (1).

The stopped trials

Trial	Ph	Sponsor	Experimental drugs	Stopped	Type	Registry stop reason
NCT03823378	2	AbbVie (ABBV)	ELSUBRUTINIB, UPADACITINIB	2020-09	Efficacy	Study M16-763 was terminated early as the benefit of each treatment arm from the feeder study (Study M16-063) did not provide appreciable evidence of differentiated clinical effect to warrant further long-term continuation.
NCT03407482	2	Roche	FENEBRUTINIB	2019-11	Efficacy	The study was ended early due to the lack of efficacy seen in the parent study GA30044.

Trial	Ph	Sponsor	Experimental drugs	Stopped	Type	Registry stop reason
NCT02628028	2	Eli Lilly LLY	POSELTINIB	2018-04	Benefit-risk	The trial was terminated after an interim analysis of efficacy and safety data did not warrant continuation of the trial.

Method

Denominator: ClinicalTrials.gov interventional Phase 2,3 immunology & autoimmune trials started 2015–2024 that have closed (completed or terminated). Numerator: terminated trials whose registry stop reason is classified as biological (efficacy, safety or benefit-risk) by Classification V2 — held-out precision 95.5%, recall 95.3% (n=600). Drugs are linked to ChEMBL and the NCI Thesaurus; 48% of industry immunology & autoimmune trials in this window carry a resolved drug target, and a trial without one cannot enter a mechanism class. Intervals are Wilson 95%.

Limits

Not a failure rate: completed trials that missed their endpoints are not in the rate. They are counted separately where the sponsor posted a primary result that can be read — most did not, so that count is a floor — and programmes discontinued after a completed trial do not appear. Stop reasons are sponsor-reported. This is a closed-trial proportion, not a time-to-event analysis: only 62.5% of trials in this segment have closed, and a trial that stops early enters the denominator sooner than one that runs to completion, which can inflate the rate in immature segments. Recent cohorts have fewer closed trials, so their rates are less stable. Research signals, not clinical or investment advice.

The whole cohort, trial by trial. The evidence package names every trial behind these figures — the ones still running and the ones that completed and missed their primary endpoint included — and places your molecule against each one that failed. €99, delivered immediately: clinicaltrialfailures.com/asset-check

Sources: ClinicalTrials.gov (NLM); ChEMBL (EMBL-EBI, CC BY-SA 3.0); NCI Thesaurus (NCI); RxNorm/RxClass (NLM); SEC EDGAR. Classification, linkage and rates by Clinical Trial Failures. Rebuilt weekly; this brief covers trials started 2015–2024.